

Anxiety - Self-Care

CCG: C-0069 (CCG)

[Link to Codes](#)**MCG Health**

Chronic Care

28th Edition

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Assessment

Initial Consideration

Q1. Patient: What would you like to talk about today? (Check all that apply.) **QM**

- Emergency or urgent problems (eg, social determinants of health, intimate partner violence, suicidal ideation)^[A] Social Determinants of Health Screening Assessment [CCG](#) Intimate Partner Violence [CCG](#) Suicide or Personal Harm Risk [CCG](#)
- Condition information
- Provider coordination (eg, next appointment, treatment clarification) Appointment Scheduling [CCG](#)
- Medications
- Complications
- Monitoring and management
- Other health problems (eg, comorbid conditions, new symptoms)
- Other, specify: _____
- Nothing
- Unknown

Q2. Patient: Do you have any changes since the last time we talked? (Check all that apply.) **QM**

- Adhering to treatment or medication problems Readiness to Change [CCG](#) Medication Adherence [CCG](#)
- Administering medication Home Infusion Management [CCG](#) Injectable Medications [CCG](#)
- Affording medicine or supplies Financial Status and Benefits [CCG](#)
- Aspiration Aspiration Risk Management [CCG](#)
- Behavioral health provider appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Blood pressure care Blood Pressure Monitoring [CCG](#)
- Blood sugar care Blood Glucose Monitoring [CCG](#) HbA1c Monitoring [CCG](#)
- Blood thinner care Anticoagulation Management [CCG](#)
- Bowel care Bowel Management [CCG](#)
- Bowel changes Bowel Symptoms [CCG](#)
- Caregiver Caregiver Resources Evaluation [CCG](#)
- Chemotherapy Chemotherapy [CCG](#)
- Cognitive impairment Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Communication or speech problems Speech and Communication Problems [CCG](#)

- Community resource [Community Resources](#)  CCG
- Dehydration care [Dehydration Monitoring and Management](#)  CCG
- Depression symptoms [Depression Screening, Adult](#)  CCG
- Dialysis care [Dialysis Management](#)  CCG
- Doctor or specialist outpatient appointment, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Energy level changes [Energy Level Changes](#)  CCG
- Enteral nutrition care [Enteral Nutrition Management](#)  CCG
- Exercising and staying active [Exercise](#)  CCG
- Fatigue or weakness [Fatigue or Weakness](#)  CCG
- Fluid restriction [Fluid Restriction](#)  CCG
- Functional status changes [Activities of Daily Living](#)  CCG [Instrumental Activities of Daily Living](#)  CCG
- Home infusion [Home Infusion Management](#)  CCG
- Hospital or emergency room visit, specify: _____ [Hospitalization Risk Assessment](#)  CCG [Post-Hospitalization Follow-Up](#)  CCG
- Medication changes [Medication Changes](#)  CCG
- Nebulizer care [Nebulizer Management](#)  CCG
- Neurologic function changes [Neurologic Function Changes](#)  CCG
- Ostomy care [Ostomy Management](#)  CCG
- Oxygen care [Oxygen Management](#)  CCG
- Pain, chronic [Chronic Pain](#)  CCG
- Pain, new or general [Pain Assessment](#)  CCG
- Parenteral nutrition care [Parenteral Nutrition Management](#)  CCG
- Peak flow meter care [Peak Flow Meter Use](#)  CCG
- Peripheral vascular (eg, lower leg redness or swelling) [Peripheral Vascular Problems](#)  CCG
- Physical impairments [Physical Impairments](#)  CCG
- Planning for long-term care needs [Advance Care Planning](#)  CCG
- Procedure, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Pulmonary treatments [Pulmonary Treatments](#)  CCG
- Rehabilitation therapy, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Respiratory changes [Respiratory Symptoms](#)  CCG
- Safety [Safety](#)  CCG
- Scheduling care [Appointment Scheduling](#)  CCG
- Sleeping problems [Sleep](#)  CCG
- Social support [Social Support](#)  CCG
- Stasis ulcer care [Stasis Ulcer Management](#)  CCG
- Testing (eg, laboratory, imaging), specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Tracheostomy care [Tracheostomy Management](#)  CCG
- Transportation [Transportation](#)  CCG
- Understanding healthcare needs [Health Literacy](#)  CCG
- Urinary catheter care [Urinary Catheter Management](#)  CCG
- Urinary changes [Urinary Symptoms](#)  CCG
- Venous thromboembolism symptoms [Venous Thromboembolism](#)  CCG
- Ventilator care [Ventilator Management](#)  CCG
- Weight gain [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG
- Weight loss [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG
- Wound care [Wound Management](#)  CCG

- Other, specify: _____
- No
- Unknown Hospitalization Risk Assessment [CCG](#)

Condition Knowledge

Q1. Patient: What type of anxiety disorder do you have? (Check all that apply.)  GAD-7 Calculator

- Generalized anxiety disorder (GAD)
- Obsessive-compulsive disorder (OCD) Obsessive-Compulsive and Related Disorders [CCG](#)
- Panic disorder
- Phobic disorder (eg, social anxiety disorder)
- Posttraumatic stress disorder (PTSD) Posttraumatic Stress Disorder [CCG](#)
- Other, specify: _____
- Unknown [P1]

Q2. Patient: What causes anxiety? (Check items understood.)

- Biochemical changes in the brain
- Exposure to traumatic events
- Family history (genetics)
- Learned behavior
- Medical problems (eg, chronic illnesses, disease exacerbation fears)
- Other behavioral health illnesses
- Stress
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q3. Case Manager: Rate the patient's understanding of the causes of anxiety. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q4. Patient: What are the symptoms of anxiety? (Check items understood.)

- Face flushing
- Fainting
- Fast breathing or dyspnea
- Fast or racing heartbeats
- Feeling restless or irritable
- Feeling sick to your stomach (nausea) or throwing up (vomiting)
- Feeling uncomfortable, apprehensive, or helpless
- High blood pressure (hypertension)
- Muscle spasms, tremors, or leg weakness
- Poor impulse control
- Problems with disorganized thoughts
- Safety-seeking behaviors
- Selective attention on sources of safety or danger
- Trouble paying attention or thinking clearly
- Unable to cope with surroundings
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q5. Case Manager: Rate the patient's understanding of the symptoms of anxiety. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q6. Patient: How can you care for anxiety to keep it from getting worse and causing problems? (Check items understood.) **QM**

- Adhere to self-care plan
- Ask for help when feelings are overwhelming
- Exercise
- Follow a daily care plan
- Follow an eating (diet) plan
- Get all follow-up laboratory tests when due
- Keep a diary about feelings, activities, eating times, and medicines taken
- Keep all doctor appointments
- Keep surroundings calm (few people, simple surroundings, dim lighting)
- Know when to contact the doctor
- Manage situation or event triggers
- Manage stress
- Seek emergency care for signs of crisis plan not working
- Seek treatment for mood disorders
- Stop using alcohol, tobacco, or drugs
- Take medicines as ordered
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q7. Case Manager: Rate the patient's understanding of how to care for anxiety. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Q8. Patient: What activities would you be able to do if you didn't have problems with your anxiety? (Check all that apply.) **QM**

- Activities of daily living (ADL) without assistance [P2]
- Cook [P2]
- Drive [P2]
- Exercise [P2]
- Go out with friends [P2]
- Go to events [P2]
- Play sports [P2]
- Play with children or grandchildren [P2]
- Shop [P2]
- Travel [P2]
- Work [P2]
- Other, specify: _____ [P2]
- Unknown [P1]

Provider Information

Q1. Patient: What doctor manages your anxiety? **QM**

- Psychiatrist or psychologist, name and phone number: _____ [Q2]
- Behavioral health therapist, name and phone number: _____ [Q3]
- Primary doctor, name and phone number: _____
- Other specialist, name and phone number: _____ [Q5]
- Don't have doctor [P1]
- Unknown [P1]

Q2. Patient: When was your last visit with the psychiatrist or psychologist?

- Less than 1 month ago
- 1 to 3 months ago
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q3. Patient: When was your last visit with a behavioral health therapist?

- Less than 1 month ago
- 1 to 3 months ago [P2]
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]
- Not applicable

Q4. Patient: When was your last visit to your primary doctor? **QM**

- Within the past 3 months
- 4 to 6 months ago
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q5. Patient: When was your last visit to the specialist doctor?

- Within the past 3 months
- 4 to 6 months ago
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q6. Patient: What other specialists have you seen within the last 12 months? (Check all that apply.) **QM**

- Behavioral health provider
- Cardiologist
- Dentist
- Dietitian
- Endocrinologist
- Gastroenterologist
- General surgeon
- Genetic counselor
- Geriatrician
- Gynecologist
- Hematologist
- Hepatologist
- HIV/AIDS
- Infectious disease
- Nephrologist
- Neurologist
- Neurosurgeon
- Obstetrician
- Oncologist
- Ophthalmologist
- Orthopedic surgeon
- Orthopedist
- Otolaryngologist
- Pain management
- Physiatrist
- Plastic surgeon
- Podiatrist
- Pulmonologist
- Rheumatologist
- Transplant specialist
- Urologist
- Other, specify: _____
- None [P1]
- Unknown [P1]

Q7. Patient: How many anxiety-related emergency department visits or hospital admissions have you had in the past 12 months? **QM**

- 2 or more [P2] Hospitalization Risk Assessment [CCG](#)
- 1 [P2]

- None
- Other, specify: _____
- Unknown [P2]

Q8. Patient: Do you have a written self-care plan from your doctor on how to care for your anxiety?[B] **QM**

- Yes [Q9]
- No [P1] [P2]
- Unknown [P1] [P2]

Q9. Patient: What are the components of your anxiety self-care plan? (Check all that apply.) **QM**

- Action plan for when symptoms start
- Attending regular therapy or counseling appointments
- Crisis management plan
- Exercise plan
- Knowing what medicines you take and why and how to take them
- Managing side effects of medicines
- Medicine use
- Nutrition plan
- Recognizing the symptoms of gradual worsening or problems
- Regular follow-up appointments for wellness checks
- When to call the doctor and when to get emergency help
- Other, specify: _____ [P1]
- Unknown [P2]
- Not applicable
- Not all appropriate information known [P2]

Q10. Case Manager: Is the patient able to understand and adhere to the self-care plan? **QM**

- Yes, independently
- Yes, with occasional reminders or caregiver support
- Yes, with frequent reminders or caregiver support
- No [P3]
- Unknown [P3]

Medication Information

Q1. Patient: Has any medicine for anxiety been prescribed by your doctor? If so, is it taken as ordered? **QM**

- Yes, and taken as ordered [Q2]
- Yes, but not taken as ordered [Q2] [P4] Medication Adherence  CCG
- No medicine prescribed [P4]
- Unknown [P4]

Q2. Patient: What medicines do you take for anxiety? (Check all that apply.) **QM**

- Anticonvulsants (eg, clonazepam, lamotrigine, valproic acid) [Q3] [P5] [P6]
- Anxiolytics (eg, alprazolam, buspirone, diazepam, meprobamate) [Q3] [P5] [P7]
- Atypical antipsychotics (eg, aripiprazole, clozapine, risperidone, ziprasidone) [Q3] [P5] [P8]
- Selective serotonin reuptake inhibitors (SSRIs) (eg, citalopram, fluoxetine, sertraline) [Q3] [P5] [P9]
- Serotonin-norepinephrine reuptake inhibitors (SNRIs) (eg, desvenlafaxine, duloxetine, venlafaxine) [Q3] [P5] [P9]
- Tricyclic antidepressants (TCAs) (eg, amitriptyline, clomipramine, trimipramine) [Q3] [P5] [P10]
- Other, specify: _____ [Q3] [P4] [P5]
- Not applicable [P4]
- Unknown [P5]

Q3. Patient: Are you having any side effects from the anxiety medicine? (Check all that apply.)

- Agitation [P5]
- Confusion [P5]
- Difficulty urinating [P5]
- Dizziness [P5]
- Fainting spells or light-headedness [P5]
- Feeling sick to your stomach (nausea) or throwing up (vomiting) [P5]
- Hallucinations [P5]
- Headache [P5]
- Inability to sleep [P5]

- Sexual problems (abnormal ejaculation, impotence, or loss of orgasm) [P5]
- Slurred speech [P5]
- Tremors or twitching [P5]
- Vision changes [P5]
- Weight loss or gain [P5]
- Other, specify: _____ [P5]
- None
- Unknown [P5]
- Not applicable

Q4. Case Manager: Rate the patient's understanding of the purpose and side effects of the medications for anxiety. **QM**

- 90% or more
- 75% to 89% [P1] [P5]
- 50% to 74% [P1] [P5]
- Less than 50% [P1] [P5]
- Unknown [P1] [P5]

Q5. Patient: Do you take any medicine that may make your anxiety worse? (Check all that apply.)

- Amphetamines [P3] [P4]
- Antihypertensives [P3] [P4]
- Appetite suppressants [P3] [P4]
- Asthma inhalants [P3] [P4]
- Cimetidine [P3] [P4]
- Cisplatin [P3] [P4]
- Decongestants [P3] [P4]
- Hay fever medicine [P3] [P4]
- Pain medicines such as meperidine [P3] [P4]
- Sinus medicines [P3] [P4]
- Other, specify: _____ [P3] [P4]
- None
- Unknown [P3] [P4]

Monitoring and Management

Q1. Patient: What problems with anxiety have you had in the past month? (Check all that apply.)

- Alcohol misuse [P2] [P3]
- Changes in behavior at work (eg, not getting work done on time, problems with coworkers, or late to work a lot) [P2] [P3]
- Changes in behavior in school (eg, being late to school a lot, changes in grades in classes, or acting out) [P2] [P3]
- Chest pain [P2] [P3]
- Dizziness or fainting [P2] [P3]
- Drug use [P2] [P3]
- Fear of being unable to escape [P2] [P3]
- Fear of dying [P2] [P3]
- Increasing anxiety or irritability [P2] [P3]
- Increasing obsessive thoughts or compulsive behaviors [P2] [P3]
- Muscle tension [P2] [P3]
- New depression or increasing depression [P2] [P3]
- Numbness or tingling [P2] [P3]
- Pounding heart [P2] [P3]
- Sleeping problems [P2] [P3]
- Trembling [P2] [P3]
- Other, specify: _____ [P2] [P3]
- None
- Unknown [P2] [P3]

Q2. Patient: Do you participate in a program for anxiety? (Check all that apply.)

- Behavioral health counseling
- Community education program
- Support group
- Exercise program
- None now, but in the past, specify: _____ [Q3] [P2] [P3]

- Other, specify: _____
- No [P2] [P3]
- Unknown [P2] [P3]

Q3. Patient: When was the last time you participated in a therapy program?

- 3 months ago or less
- 4 to 11 months ago
- 12 months ago or more
- Other, specify: _____ [P2]
- Unknown [P2]
- Not applicable

Q4. Patient: Do you follow a special diet? (Check all that apply.) **QM**

- Caffeine restriction [P2]
- DASH (low sodium) Diet Knowledge Insufficient - DASH Diet [CCG](#)
- Heart healthy Diet Knowledge Insufficient - Heart Healthy [CCG](#)
- Kidney diet Diet Knowledge Insufficient - Renal Dysfunction [CCG](#)
- Low carb or low glycemic index Diet Knowledge Insufficient - Low Glycemic Index [CCG](#)
- Other, specify: _____ Nutritional Status, Adult [CCG](#)
- No [P2]
- Unknown [P2]

Q5. Patient: Do you get regular exercise? **QM**

- Yes
- No [P2]
- Unknown [P2]

Q6. Patient: Is there someone you talk to about your problems?

- Yes, name and relationship: _____
- No [B1] [P11]
- Unknown [B1] [P11]

Q7. Patient: Do you have someone to count on when you need help?

- Yes, name and relationship: _____
- No [B1] [P11]
- Unknown [B1] [P11]

Q8. Patient: Is there someone who can help you feel better when you are stressed, sad, or upset?

- Yes, name and relationship: _____
- No [B1] [P11]
- Unknown [B1] [P11]

Q9. Patient: Is there someone who accepts you for who you are, the good and bad?

- Yes, name and relationship: _____
- No [B1] [P11]
- Unknown [B1] [P11]

Q10. Patient: Is there someone you can count on to care about you no matter what is happening?

- Yes, name and relationship: _____
- No [B1] [P11]
- Unknown [B1] [P11]

Q11. Patient: Does your social support help you as much as you need? **QM**

- Yes
- No [B1] [P11]
- Unknown [B1] [P11]

Q12. Patient: Has your mood changed over the past 2 weeks?(1)(2)  PHQ-2 Calculator **QM**

- Yes [Q13]
- No
- Unknown

Q13. Patient: What changes in your mood have you had? (Check all that apply.) **QM**

- Change in usual activity (eg, working excessively, no longer talking to friends, or going on shopping sprees) Depression Screening, Adult [CCG](#)
- Down, hopeless, or depressed Depression Screening, Adult [CCG](#)
- Forgetting a lot or getting angry or frustrated Depression Screening, Adult [CCG](#)
- Little interest or pleasure in doing things Depression Screening, Adult [CCG](#)
- Quick mood changes Depression Screening, Adult [CCG](#)
- Other, specify: _____ Depression Screening, Adult [CCG](#)
- Not applicable

Q14. Patient: Are there any current sleeping problems? (Check all that apply.) **QM**

- Falling asleep [P12]
- Staying asleep [P12]
- Waking up too early [P12]
- Not feeling rested after sleeping [P12]
- Other, specify: _____ [P12]
- None
- Unknown [P12]

Q15. Patient: What do you do to help sleep? (Check all that apply.)

- Decrease caffeine
- Don't eat big meals before bedtime
- Get daily exercise
- Go to bed and get up at routine times of the day
- Keep journal of thoughts
- Sleep in bed, not on chair or sofa
- Sleep in dark, quiet room
- Sleep with head elevated, not lying flat
- Stopped drinking
- Stopped smoking
- Take a warm bath or shower before bedtime
- Take medicines, specify: _____ [Q16]
- Took distractions out of bedroom (eg, TV)
- Use equipment (eg, oxygen or CPAP)
- Other, specify: _____
- Nothing
- Unknown [P12]
- Not all appropriate sleep promoting activities being used [P12]

Q16. Patient: Do the medicines work to help sleep?

- Yes
- No [P12]
- Unknown
- Not applicable

Q17. Patient: Are you having any problems with your anxiety self-care needs? (Check all that apply.) **QM**

- Affording medicine or supplies [P2] [P3] Financial Status and Benefits [CCG](#)
- Behavior indicating cognitive impairment [P2] [P3] Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Caregiver help needed [P2] [P3] Caregiver Resources Evaluation [CCG](#)
- Community resource information needed [P2] [P3] Community Resources [CCG](#)
- Daily care of child [P2] [P3] Caregiver Resources Evaluation [CCG](#)
- Exercising and staying active [P2] [P3] Exercise [CCG](#)
- Safety in the home [P2] [P3] Safety [CCG](#)
- Taking medicine as prescribed [P2] [P3] Medication Adherence [CCG](#)
- Transportation for care [P2] [P3] Transportation [CCG](#)
- Trouble gaining weight [P2] [P3] Nutritional Status, Adult [CCG](#) Nutritional Management to Gain Weight [CCG](#)

- Trouble losing weight [P2] [P3] Nutritional Status, Adult [CCG](#) Nutritional Management to Lose Weight [CCG](#)
- Trouble making lifestyle changes [P2] [P3] Readiness to Change [CCG](#)
- Trouble understanding how to do self-care [P2] [P3] Health Literacy [CCG](#)
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P2] [P3]

Q18. Case Manager: Rate the patient's understanding of the monitoring and management of anxiety. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Other Health Considerations

Q1. Patient: Do you use tobacco, alcohol, or drugs? (Check all that apply.) **QM**

- Yes, tobacco use Tobacco Use [CCG](#)
- Yes, alcohol use Alcohol Use [CCG](#)
- Yes, drug use Substance Use [CCG](#)
- No
- Unknown Tobacco Use [CCG](#) Alcohol Use [CCG](#) Substance Use [CCG](#)

Q2. Patient: Do you have other conditions that might make anxiety worse? (Check all that apply.) **QM**

- Alcohol or substance misuse [P2] [P3] Substance-Related Disorders - Self-Care [CCG](#) Substance Use [CCG](#)
- Attention-deficit hyperactivity disorder (ADHD) [P2] [P3] Attention-Deficit Hyperactivity Disorder, Adult - Self-Care [CCG](#) Attention-Deficit Hyperactivity Disorder, Pediatric - Self-Care [CCG](#)
- Autism spectrum disorder [P2] [P3]
- Bipolar disorder [P2] [P3] Bipolar Disorder - Self-Care [CCG](#)
- Cancer [P2] [P3] Cancer - Self-Care [CCG](#)
- Chronic obstructive pulmonary disease (COPD) [P2] [P3] Chronic Obstructive Pulmonary Disease - Self-Care [CCG](#)
- Coronary artery disease [P2] [P3] Coronary Artery Disease - Self-Care [CCG](#)
- Depression [P2] [P3] Depression - Self-Care [CCG](#)
- Heart attack [P2] [P3]
- Heart failure [P2] [P3] Heart Failure - Self-Care [CCG](#)
- Heart valve disease [P2] [P3]
- High blood pressure [P2] [P3] Hypertension - Self-Care [CCG](#)
- Hyperthyroidism [P2] [P3]
- Obesity or overweight [P2] [P3] Obesity, Adult - Self-Care [CCG](#)
- Physical inactivity [P2] [P3] Exercise [CCG](#)
- Posttraumatic stress disorder (PTSD) [P2] [P3] Posttraumatic Stress Disorder [CCG](#)
- Pregnancy [P2] [P3] High-Risk Pregnancy - Self-Care [CCG](#)
- Thyroid disease [P2] [P3]
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P1]

Q3. Patient: What vaccinations have you had? (Check all that apply.) **QM**

- Coronavirus disease 2019 (COVID-19) [Q4]
- *Haemophilus influenzae* type b (Hib) [Q4] [Q5]
- Hepatitis A [Q4]
- Hepatitis B [Q4]
- Human papillomavirus (HPV) [Q4]
- Influenza [Q4] [Q5]
- Measles, mumps, rubella (MMR) [Q4]
- Meningococcal 4-valent conjugate (MenACWY) or polysaccharide (MPSV4) [Q4]

- Meningococcal B (MenB) [Q4]
- Pneumococcal 13-valent conjugate (PCV13) [Q4] [Q6]
- Pneumococcal 15-valent conjugate (PCV15) [Q4] [Q6]
- Pneumococcal 20-valent conjugate (PCV20) [Q4] [Q6]
- Pneumococcal 23-valent polysaccharide (PPSV23) [Q4] [Q6]
- Polio (inactivated) [Q4]
- Respiratory syncytial virus (RSV) [Q4]
- Rotavirus [Q4]
- Tetanus booster [Q4]
- Tetanus, diphtheria, pertussis (Td/Tdap) [Q4]
- Varicella [Q4]
- Zoster [Q4]
- Other, specify: _____ [Q4]
- None, because: _____ [P13]
- Unknown [P13]

Q4. Patient: Are you up to date with the vaccinations you need (for age, booster status, or medical condition)? **QM**

- Yes
- No, because: _____ [Q7] [P13]
- Unknown [Q7] [P13]

Q5. Patient: When was your last flu shot? **QM**

- In the past year
- More than a year ago [Q7] [P13]
- Never [Q7] [P13]
- Unknown [Q7] [P13]

Q6. Patient: When was your last pneumonia shot?

- In the last year
- 2 to 4 years ago
- 4 to 5 years ago [Q7] [P13]
- More than 5 years ago [Q7] [P13]
- Never [Q7] [P13]
- Unknown [Q7]

Q7. Patient: Do you have an appointment scheduled to get caught up on your vaccinations?

- Yes
- No [P13]
- Unknown [P13]

Q8. Case Manager: Rate the patient's understanding of the potential complications of anxiety. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Patient Understanding Evaluation

Q1. Case Manager: Does the patient understand all the causes of anxiety?

- Yes
- No [P1]
- Unknown [P1]

Q2. Case Manager: Does the patient understand all the symptoms of anxiety?

- Yes
- No [P1]
- Unknown [P1]

Q3. Case Manager: Does the patient understand how to care for anxiety? **QM**

- Yes
- No [P1] [P2]

- Unknown [P1] [P2]

Q4. Case Manager: Does the patient know when and where to get help for problems?

- Yes
- No [P1] [P3]
- Unknown [P1] [P3]

Q5. Case Manager: Does the patient have the ability to understand and follow the self-care plan? **QM**

- Yes
- No [P3]
- Unknown [P3]

Q6. Case Manager: Does the patient understand the purpose and side effects of their anxiety medication?

- Yes
- No [P5]
- Unknown [P5]

Q7. Case Manager: Is the patient having trouble coping with their anxiety? **QM**

- Yes [P3]
- No
- Unknown [P3]

Q8. Case Manager: Does the patient agree to education? **QM**

- Yes
- No, explain why: _____ [P3] Readiness to Change  CCG
- Unknown [P3]

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.
- HEDIS Measures include(3):
 - Tobacco use is assessed and addressed.
 - Readmission risk is assessed and addressed.
 - Alcohol and substance misuse is assessed and addressed.

Plan of Care

Barriers

B1. Inadequate social support

Problems

P1. Anxiety information needed

- **Goal:** Anxiety diagnosis better understood(4)(5)(6) 
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Definition(7)(8):**
 - Emotional disorder of distress with intense and potentially disabling anxiety reactions
 - Most common of all behavioral health illnesses
 - Types include generalized anxiety disorder (GAD), obsessive-compulsive disorder (OCD), panic disorder, phobic disorder (eg, social phobia), and posttraumatic stress disorder (PTSD).
 - Occurs in absence of threatening situation (unlike fear) and can be response to perceived future threat
 - Chronic (lifelong) in nature and interferes with life functions (eg, work, school)
 - **EDUCATE: Causes(8)(9):**
 - Biochemical influences
 - Family history (genetics)
 - Psychosocial and environmental influences
 - Traumatic event history
 - **EDUCATE: Symptoms(8):**

- Concentration difficulty
- Coping problems
- Emotional difficulties
- Face flushing
- Fainting
- Heartbeat fast or irregular
- Hypertension
- Impulse control problems
- Muscle cramps, spasms, or weakness
- Nausea or vomiting
- Agitation
- Social changes (eg, isolation, withdrawal)
- Dyspnea
- Thought process problems (eg, planning, reasoning, decision making)
- **EDUCATE: Diagnosis:**
 - Based on medical and behavioral history, physical examination, and diagnostic tests
- **EDUCATE: Treatment(7):**
 - Medication management
 - Comorbid condition management
 - Psychotherapy (eg, cognitive behavioral therapy (CBT), individual or group therapy, family therapy)
 - Symptom management
- **EDUCATE: Treatment goals:**
 - Complication and progression management(9) 
 - Functional improvement and maintenance (eg, ADL, IADL)
 - Psychosocial functioning improved and maintained
 - Quality of life improvement
 - Remission
 - Safety (for self and others)
 - Symptom frequency and severity reduced
- **EDUCATE: Resources:**
 - National Alliance on Mental Illness: www.nami.org
 - National Institute of Mental Health: www.nimh.nih.gov
 - National Suicide Prevention Lifeline: www.988lifeline.org
 - Smokefree. National Institutes of Health: www.smokefree.gov
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Social worker referral, as needed(10)
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Care team meeting, if needed(11)(12)
- **COORDINATE:** Communication between care team members and care plan reconciliation(11)(13)(14)
- **COORDINATE:** Follow-up to evaluate anxiety understanding, referral status, and need for further assistance(15)(16)
- **SEND:** Your care plan - Anxiety  (Spanish)
- **SEND:** What you need to know - Anxiety  (Spanish)
- **SEND:** When to get help - Anxiety  (Spanish)
- **SEND:** Daily care for your condition - Anxiety  (Spanish)
- **SEND:** Be ready for your doctor visit - Anxiety  (Spanish)
- **SEND:** Be ready with questions for your doctor - Anxiety  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)

P2. Anxiety self-care needed

- **Goal:** Anxiety self-care plan developed, implemented, and maintained(4)(5)(17)(18) 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help):
 - Development of crisis plan
 - Support system (eg, family, friends) identification, enlistment, and education
 - **ASSIST:** Communication between multiple providers(19)

■ **EDUCATE: Self-care plan information(8):**

- Action plan (eg, when to initiate relaxation, cognitive, and stress-management techniques)
- Adherence to plan necessary to achieve goals, and complications of nonadherence (eg, exacerbations, progression, comorbidities)
- Behavioral health treatment (eg, cognitive behavioral therapy (CBT), individual therapy, family therapy)(9)(20)(21)
- Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue.(21)
- Comorbid condition management
- Daily care (eg, symptom management)
- Set small goals every day (eg, get out of bed at a certain time, finish one chore or project, or do something that brings pleasure like listen to music).(22)
- Exercise program within provider's recommendations(23)(24)
- Functional changes care (eg, adaptations and support for ADL and IADL)(24)
- Health checks, diary, and action plans(25)
- Medication purpose, use, effects, and considerations
- Diet considerations (eg, reduce caffeine, improve diet quality)(26)(27)
- Relapse signs (eg, insomnia, decreased appetite, poor personal hygiene)(8)
- Risk factor management (eg, trigger avoidance)(9)
- Safety awareness and management (eg, remove suicide risks, de-escalation techniques)
- Sleep hygiene (eg, regular pattern, calm sleep environment)(8)
- Stress management (eg, relaxation and coping techniques)(9)
- Substance use avoidance(7)
- Support group participation
- Symptom management (eg, listen to music, exercise, mindfulness, massage, biofield therapy)(8)(9)
- Tobacco cessation plan
- Wellness checkups (eg, vaccinations, screenings)

■ **EDUCATE: When to get emergency help(22):**

- Suicidal or homicidal actions or plans(28)
- Crisis plan not working
- Hallucinations or delusions, increased or new
- Psychotic behavior (eg, threatening)
- Consciousness level change, abrupt
- Seizures, new or complicated (eg, seizure longer than 5 minutes, multiple seizures with loss of consciousness, head injury during seizure)(29)
- Heart attack or stroke signs

■ **EDUCATE: When to call the provider for behavior changes(9)(27):**

- Appointments missed (eg, support group, provider)
- Depression (eg, feeling sad, hopeless, overwhelmed)(22)(30)
- Impulse control problems
- Mood changes (eg, depression, anger, agitation)
- Mood instability (eg, extreme or rapid changes)
- Self-care changes (eg, ADL or IADL problems)
- Medication problems (eg, side effects, access, administration)
- Substance misuse (eg, increased use, use interferes with ADL or IADL)

■ **EDUCATE: When to call the provider(27):**

- Bleeding problems (eg, unusual bruising or bleeding)
- Emergency department or urgent care visit
- Fainting
- Falls
- Headache, prolonged or unresponsive to treatment
- Medication problems (eg, side effects, access, administration)
- Medications newly ordered from other providers
- Nausea, persistent
- Pregnant or planning to get pregnant
- Urinary problems (eg, pain, frequency change, blood)(31)
- Visual changes, new-onset

■ **COORDINATE: Tobacco use treatment, as needed**

■ **COORDINATE: Behavioral health services referral, if indicated[D]**

■ **COORDINATE: Treatment program referral, if indicated(20)(32)**

■ **COORDINATE: Crisis services referral, as needed**

■ **COORDINATE: Social worker referral, as needed(10)**

- **COORDINATE:** Community services referral, as needed(33)
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Telehealth referral, if indicated[E]
- **COORDINATE:** Vocational rehabilitation referral, as needed(34)
- **COORDINATE:** Caregiver health service referral, as needed
- **COORDINATE:** School or education support, as needed
- **COORDINATE:** Care team meeting, if needed(11)(12)
- **COORDINATE:** Communication between care team members and care plan reconciliation(11)(13)(14)
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(15)(16)
- **SEND:** Your care plan - Anxiety  (Spanish)
- **SEND:** What you need to know - Anxiety  (Spanish)
- **SEND:** When to get help - Anxiety  (Spanish)
- **SEND:** Daily care for your condition - Anxiety  (Spanish)
- **SEND:** Be ready for your doctor visit - Anxiety  (Spanish)
- **SEND:** Be ready with questions for your doctor - Anxiety  (Spanish)
- **SEND:** Caregiver tips  (Spanish)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Help with sleeping or insomnia  (Spanish)
- **SEND:** How to stop smoking  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** Symptoms diary (Illustrated)  (Spanish)

P3. Anxiety potential complications and comorbidities monitoring needed

- **Goal:** Anxiety potential complications and comorbidities evaluated and addressed(4)(5)[N](#)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of problems, comorbidities, and self-care plan[F]:
 - Recently hospitalized patient's evaluation within 1 week of discharge[N](#)
 - Recently hospitalized patient's evaluation with outpatient healthcare providers (eg, nurse, social worker) within 1 to 3 days of discharge(15)
 - Appointment every 3 to 6 months when patient is symptom free(20)
 - Appointment once a month or more for patient with questionable adherence to treatment plan
 - **EDUCATE:** When and how to get help for worsening anxiety problems
 - **EDUCATE:** Treatment and monitoring for comorbid conditions (eg, coronary artery disease, high blood pressure, heart valve disease)(27)
 - **EDUCATE:** Anxiety care can help improve outcomes of comorbid conditions.
 - **EDUCATE:** Medications prescribed for anxiety may worsen chronic conditions.(27)
 - **EDUCATE:** Perinatal effects of condition and medication
 - **EDUCATE:** Follow-up appointments for assessment of clinical status[G]
 - **COORDINATE:** Specialist referral, if indicated
 - **COORDINATE:** Treatment program referral, if indicated[H]
 - **COORDINATE:** Preconception specialist referral, as needed
 - **COORDINATE:** Communication between care team members and care plan reconciliation(11)(13)(14)
 - **COORDINATE:** Follow-up contact to review self-care plan, referral status, and need for further assistance(15)(16)
 - **SEND:** Be ready for your doctor visit - Anxiety  (Spanish)
 - **SEND:** Be ready with questions for your doctor - Anxiety  (Spanish)
 - **SEND:** Narcotic medicine side effects  (Spanish)
 - **SEND:** Pain medicines - Addiction and withdrawal  (Spanish)
 - **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
 - **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)

P4. Anxiety medication evaluation needed

- **Goal:** Anxiety medication evaluation completed and medication regimen initiated, if indicated

- **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for evaluation of condition status
- **EDUCATE:** When and how to get help for worsening anxiety problems
- **EDUCATE:** Medication management needs common for anxiety problems
- **COORDINATE:** Follow-up after provider contact for evaluation of medication changes(15)(16)

P5. Anxiety medication management needed

- **Goal:** Medication information understood(35)

- **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
- **ASSIST:** Providing reconciled medication list to all providers. See Medication Reconciliation Tool  SR for more information.
- **EDUCATE: Purpose:** Importance of understanding what each medication is for, how it should be taken, and what to do if dose is missed
- **EDUCATE:** Medication indications, side effects, and considerations frequently change. See a drug reference for specific information not listed here.
- **EDUCATE:** Administration techniques:
 - Route (eg, oral, intramuscular (IM), intravenous (IV))
 - With or without foods or liquids (eg, milk)
 - Timing of administration
 - Appropriate measuring device (eg, liquid medications)
 - Proper storage
- **EDUCATE: Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
- **EDUCATE: Side effects:** Management, and when to contact provider
 - Common side effects include gastrointestinal changes, dry mouth, fatigue, insomnia, rash, dizziness, and headache.
- **EDUCATE: Considerations:**
 - Discuss medication with provider if pregnant, planning a pregnancy, or breast-feeding.
 - Comorbidities may be contraindications to the use of a drug (eg, liver disease, diabetes).
 - Health checks, diary, and action plans (eg, blood pressure, weight, heart rate)
 - Safety measures (eg, sun protection, driving, or operating machinery)
 - Contact provider or pharmacist prior to taking any other over-the-counter or prescribed medication.
- **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)
- **COORDINATE:** Communication between care team members and medication list reconciliation
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(15)(16)
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Therapeutic drug level monitoring
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P6. Anticonvulsant medication management

- **Goal:** Anticonvulsant medication initiated and maintained(22)(36)

- **Interventions:**

- **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
- **EDUCATE: Purpose:** Anticonvulsants may be used as mood stabilizers or for seizure treatment.
- **EDUCATE: Call provider immediately:** Vision problems, burning or tingling sensation, confusion, slowing of mental and physical ability, memory problems, speech problems, unusual fatigue, bleeding problems
- **EDUCATE: Side effects:** Appetite changes, tremors
- **EDUCATE: Considerations:**
 - Obtain baseline platelet count and bleeding time before starting valproic acid.

- Valproic acid is not advisable for women of childbearing age due to the risk of birth defects.

- **SEND:** Anticonvulsant medicines  (Spanish)

P7. Anxiolytic medication management needed

- **Goal:** Anxiolytic medication initiated and maintained(27)(37)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Anxiolytics are used for treating acute anxiety and panic disorders.
 - Optimal effect may take 2 weeks.
 - Used as a short-term or occasional-use agent due to side effects and physical dependence concerns
 - **EDUCATE: Call provider immediately:** Slurred speech; staggering; respiratory depression; severe depression, weakness, vertigo, or confusion
 - **EDUCATE: Side effects:** Drowsiness, motor function impairment, urinary changes, confusion, vision changes, agitation
 - Withdrawal symptoms may occur when discontinued (eg, depression, insomnia, anxiety, muscle cramps, tremors, vomiting, sweating, seizures, delirium).
 - **EDUCATE: Considerations:** Use cautiously with glaucoma.
 - **SEND:** Safety in the home - Fall protection  (Spanish)
 - **SEND:** Safety in the home - Fire and other risks  (Spanish)

P8. Atypical antipsychotic medication management needed

- **Goal:** Atypical antipsychotic medication initiated and maintained
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Atypical antipsychotics may be indicated for rapid control of acute mania to decrease agitation, aggression, and dangerous behavior.(38)(39)
 - Second-generation antipsychotic medication
 - May be indicated for second-line, third-line, or adjunctive therapies(40)
 - **EDUCATE: Call provider immediately:** Neuroleptic malignant syndrome (NMS) (eg, extreme muscle rigidity, high fever, tachycardia, tachypnea, diaphoresis, stupor, fluctuations in blood pressure)(41)
 - **EDUCATE: Side effects:** Difficulty urinating; jaundice; shuffling walk; fainting spells; seizures; pharyngitis; muscle spasms; excitement or restlessness; jerky movements of head, face, or neck; tachycardia; weight gain; blurred vision(42)
 - Increased risk of metabolic side effects, including obesity, diabetes, and dyslipidemia(39)
 - **EDUCATE: Considerations(39):**
 - Use cautiously with history of seizures, liver disease, heart disease, blood disorders, or pregnancy.(26)
 - Monitor blood glucose, cholesterol, and triglyceride levels.
 - Talk to provider before discontinuing medication.
 - **COORDINATE:** Electrocardiogram (ECG) monitoring, if indicated(39)
 - **COORDINATE:** Laboratory testing

P9. Selective serotonin reuptake inhibitors (SSRIs) or serotonin-norepinephrine reuptake inhibitors (SNRIs) medication management needed

- **Goal:** Selective serotonin reuptake inhibitors (SSRIs) or serotonin-norepinephrine reuptake inhibitors (SNRIs) medication initiated and maintained
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** SSRIs and SNRIs are antidepressant medications.(43)
 - Prevent anxiety symptoms, reduce relapse risk, and have less safety risks compared to benzodiazepines(44)
 - Effects may take several weeks.(45)
 - **EDUCATE: Call provider immediately:** If taking SSRIs and MAOIs or TCAs and serotonin syndrome signs present(46) (47):
 - Serotonin syndrome (eg, tachycardia, diarrhea, hyperthermia, tremor, clonus, agitation, delirium)
 - **EDUCATE: Side effects:** Agitation, headache, postural hypotension, sexual dysfunction (eg, abnormal ejaculation, impotence, loss of orgasm)(48)
 - SNRIs have fewer sexual side effects than SSRIs.
 - **EDUCATE: Considerations(48)(49):**
 - Take dose early in the day to help avoid insomnia at night.
 - Use cautiously with cardiac conditions due to possible elevation in blood pressure with SNRIs.
 - **SEND:** Help with sleeping or insomnia  (Spanish)
 - **SEND:** Antidepressant medicines  (Spanish)

P10. Tricyclic antidepressants (TCAs) medication management needed

- **Goal:** Tricyclic antidepressants (TCAs) medication initiated and maintained

- o **Interventions:**

- **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
- **EDUCATE: Purpose:** TCAs are antidepressant medications that decrease the frequency and intensity of panic attacks.(49)
 - Clomipramine may be indicated for obsessive-compulsive disorder.(50)
- **EDUCATE: Call provider immediately:** If taking TCAs and MAOIs or SSRIs and serotonin syndrome signs present(46)(47):
 - Serotonin syndrome (eg, tachycardia, diarrhea, hyperthermia, tremor, clonus, agitation, delirium)
- **EDUCATE: Side effects:** Blurred vision, urinary retention, orthostatic hypotension, reduction in seizure threshold, tachycardia, light sensitivity, weight gain
- **EDUCATE: Considerations:** Avoid in patients with angle-closure glaucoma or urinary retention.(51)

P11. Social support needed

- **Goal:** Social support improved(52)(53)N

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Patient in establishment of a helpful social network(54):
 - Identification of individuals in support network
 - Engaging in social support system (gathering information, helping patient develop skills, encouraging shifts in perspective, and allowing patient to vent about issues)
- **EDUCATE:** Ways to access social support
- **EDUCATE:** Role of social support in quality of life (eg, improved self-image, coping)(53)
- **EDUCATE:** Patient's role in social support relationships (eg, be aware of how actions impact the feelings of family and friends)(55)
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Community services referral, as needed(56)
- **COORDINATE:** Social worker referral, as needed(10)
- **COORDINATE:** Support group referral, as needed(57)(58)
- **COORDINATE:** Support network contact (eg, church, social groups), as needed(54)
- **COORDINATE:** Follow-up to evaluate social support, referral status, and need for further assistance(15)(16)

P12. Sleeping problems present

- **Goal:** Sleep improvedN

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Scheduling provider appointment for medical evaluation of sleep changes and creation of self-care plan to include(59)(60):
 - Identifying techniques to improve sleep quality(61)
 - Medication changes that can help patient sleep better (eg, changing the time stimulants are taken to early in the day or not taking diuretic before bedtime)
 - Review of herbal preparations that may promote sleep(62)
 - Creation of sleep diary(63)
- **EDUCATE:** Sleep hygiene techniques(59)(61)(64):
 - Schedule sleep (go to bed and get up at same time every day).
 - Spend no more than 8 hours per day in bed. Amount of average sleep time for adult is 7 to 8 hours.
 - Limit napping to once a day for 20 to 30 minutes in early afternoon.
 - Sleep in a dark, quiet bedroom.
 - Use bedroom only for sleeping, not to watch TV, work, use electronics, or do other activities.
 - Avoid pets in the bed.
 - Get safe amount of sunlight every day and avoid bright lights before bedtime.
 - Cut down on drinking fluids in the afternoon if nighttime urination is a problem.
 - Avoid substances that may influence sleep (eg, caffeine, stimulants, alcohol).
 - Stop smoking; nicotine is a stimulant.
 - Get regular exercise, but avoid vigorous exercise 2 hours before bedtime.(65)
 - Write down urgent thoughts if mind-racing is a problem.
 - Do not eat big meals before bedtime; gastric process can cause sleep problems.
- **EDUCATE:** Herbal preparations to help sleep (eg, melatonin, tryptophan, valerian), as approved by provider(66)N

- **EDUCATE:** Behavioral health treatment options and benefits (eg, cognitive behavioral therapy for insomnia)(63)(67)^N
- **EDUCATE:** Hypnotic medications use and misuse, if prescribed for insomnia(60)
- **COORDINATE:** Sleep specialist referral, if indicated(66)
- **COORDINATE:** Behavioral health services referral, if indicated(63)
- **COORDINATE:** Communication between care team members and care plan reconciliation(11)(13)(14)
- **COORDINATE:** Follow-up contact to evaluate sleep, status of referrals, and need for further assistance(15)(16)
- **SEND:** Help with sleeping or insomnia  (Spanish)

P13. Vaccinations not up to date, increasing infection risk

- **Goal:** Vaccinations up to date(68)(69)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Identifying vaccines patient may need that are not contraindicated based on clinical status
 - **EDUCATE:** Purpose and benefits of vaccinations
 - **EDUCATE:** Recommended immunizations and timing of administration:
 - Centers for Disease Control and Prevention's vaccination schedule: www.cdc.gov/vaccines(70)
 - Vaccine schedules may need to be altered based on medications; check with the specialist provider for information.(71)
 - Possible side effects or complications associated with getting vaccinated
 - **EDUCATE:** Influenza vaccination annually(72):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects with shot are sore, red, or swollen area where shot was given; fever; and aches.
 - Common side effects with nasal spray are mild and last 1 or 2 days; they include runny nose, headache, sore throat, and cough.
 - Allergic reaction rare and is usually caused by a chicken egg allergy
 - **EDUCATE:** Pneumococcal vaccination potential side effects:
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; fever; and muscle aches.
 - Serious side effects are rare.
 - **EDUCATE:** COVID-19 vaccination potential side effects(70):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; chills; fatigue; and aches.
 - Serious side effects are rare.
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Vaccinations
 - **COORDINATE:** Follow-up to evaluate vaccination understanding, referral status, and need for further assistance(15)(16)
 - **SEND:** Coronavirus disease 2019 (COVID-19) vaccines  (Spanish)
 - **SEND:** Flu vaccine  (Spanish)
 - **SEND:** Pneumonia vaccine for adults  (Spanish)
 - **SEND:** Hepatitis A vaccine  (Spanish)
 - **SEND:** Hepatitis B vaccine  (Spanish)

Evidence Summary

Quality standards: The Case Management Society of America's Standards of Practice for Case Management principles include facilitation of informed decision making, counseling, and health education with patient self-care that is responsive to the patient's preferences (eg, culture, values, needs).(73)

Evidence: In pediatric longitudinal samples, having separation, social, or generalized anxiety disorder doubles or triples the subsequent risk of developing one of the other two disorders before age 20 years.(74)

Quality standards: Standards indicate that program content should include an individualized self-care plan that includes patient preferences, prioritized goals, interventions with information on whom to contact in urgent situations, and scheduled follow-up. Programs should facilitate communication and collaboration with practitioners in a timely manner.(73)(75)(76)(77)

Evidence: Systematic reviews and meta-analyses of randomized controlled trials have found psychotherapy (eg, cognitive behavioral therapy (CBT)) to have demonstrated efficacy for anxiety disorders in adults, children, and adolescents.(24)

Evidence: The National Institute for Health and Care Excellence (NICE) recommends the use of individual cognitive behavioral therapy (CBT) as first-line treatment in adults for the treatment of social anxiety disorder.(32)

National guidelines: The 2018 Physical Activity Guidelines Advisory Committee advises that adults with chronic conditions or disabilities, who are able, should follow the key activity guidelines for adults. The guidelines recommend that adults should do at least 150 to 300 minutes a week of moderate-intensity or 75 to 150 minutes a week of vigorous-intensity aerobic physical activity, or an equivalent combination of moderate-intensity and vigorous-intensity aerobic activity. They should also do muscle-strengthening activities on 2 or more days a week. Older adults should do multicomponent physical activity that includes balance training as well as aerobic and muscle-strengthening activities.(78)

Quality standards: Standards indicate that programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(73)(75)(76) (77)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(79)(80)(81) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(15) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(82)(83)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication). (84)

Quality standards: Standards indicate that program content should address patient supports and psychosocial issues that may impede self-management. Programs should have a process for following up with patients on the status of referrals.(73)

Quality standards: Standards indicate that program content should include a condition treatment plan, self-management and self-monitoring, and health behaviors (eg, sleep patterns). Programs should have a process for following up with patients on the status of referrals.(73)

Specialty society: In a systematic review of the pharmacologic treatment of chronic insomnia in adults, the American Academy of Sleep Medicine does not recommend diphenhydramine, melatonin, tryptophan, or valerian for treating sleep onset or maintenance.(62)

Specialty society: The American College of Physicians recommends cognitive behavioral therapy for insomnia as the first-line treatment for patients with chronic insomnia (trouble falling or staying asleep for 3 or more nights a week for at least 3 months, with daytime consequences). Cognitive behavioral therapy for insomnia generally includes cognitive therapy with sleep restriction, relaxation training, and stimulus control for 5 to 8 sessions.(85)

References

- Chiappetta L. Mood disorders: depression. In: Fosbre CD, Varcrolis EM, Chiappetta L, editors. Varcrolis' Essentials of Psychiatric-Mental Health Nursing. 5th ed. Elsevier; 2023:207-234. [Context Link 1]
- US Preventive Services Task Force, et al. Screening for Depression and Suicide Risk in Adults: US Preventive Services Task Force Recommendation Statement. Journal of the American Medical Association 2023;329(23):2057-2067. DOI: 10.1001/jama.2023.9297. (Reaffirmed 2023 Jul 06) [Context Link 1] View abstract...
- HEDIS quality measures 2023. [Internet] NCQA. Accessed at: <https://www.ncqa.org/hedis/measures/>. Updated 2023 [accessed 2023 Oct 23] [Context Link 1]
- Assessing the adult patient through application of the integrated case management complexity assessment grid. In: Fraser K, Perez R, Latour C, editors. CMSA's Integrated Case Management. New York, NY: Springer Publishing Company LLC; 2018:51-82. [Context Link 1, 2, 3]
- Assessing the pediatric patient using the integrated case management complexity assessment grid. In: Fraser K, Perez R, Latour C, editors. CMSA's Integrated Case Management. New York, NY: Springer Publishing Company LLC; 2018:83-102. [Context Link 1, 2, 3]
- Improving adherence. In: Perez R, Rogers S, Prince M, Fraser K, editors. Case Management Adherence Guidelines. 2020 ed. Case Management Society of America; 2022:19-42. [Context Link 1]
- Generalised Anxiety Disorder And Panic Disorder In Adults: Management. NICE Clinical Guideline CG113 [Internet] National Institute for Health and Care Excellence. 2020 Jun Accessed at: <https://www.nice.org.uk/guidance>. [created 2011; accessed 2023 Aug 08] [Context Link 1, 2, 3]
- Anxiety and anxiety disorders. In: Videbeck SL, editor. Psychiatric-Mental Health Nursing. 9th ed. Wolters Kluwer; 2023:222-240. [Context Link 1, 2, 3, 4, 5, 6, 7, 8]
- Chiappetta L. Anxiety, anxiety disorders, and obsessive-compulsive and related disorders. In: Fosbre CD, Varcrolis EM, Chiappetta L, editors. Varcrolis' Essentials of Psychiatric-Mental Health Nursing. 5th ed. Elsevier; 2023:138-160. [Context Link 1, 2, 3, 4, 5, 6, 7]
- McQueen A, et al. Social needs, chronic conditions, and health care utilization among Medicaid beneficiaries. Population Health Management 2021;24(6):681-690. DOI: 10.1089/pop.2021.0065. [Context Link 1, 2, 3] View abstract...
- Case management standards and professional organizations. In: Powell SK, Tahan H, editors. Case Management a Practical Guide for Education and Practice. 4th ed. Philadelphia, PA: Wolters Kluwer, Lippincott, Williams & Wilkins; 2019:314-354. [Context Link 1, 2, 3, 4, 5, 6]

12. Ralston JD, Wagner EH. Comprehensive chronic disease management. In: Goldman L, Schafer AI, editors. *Goldman-Cecil Medicine*. 26th ed. Elsevier; 2020:42-45.e2. [Context Link 1, 2]
13. Karam M, et al. Nursing care coordination for patients with complex needs in primary healthcare: a scoping review. *International Journal of Integrated Care* 2021;21(1):16. DOI: 10.5334/ijic.5518. [Context Link 1, 2, 3, 4] View abstract...
14. Joo JY, Liu MF. The experience of chronic illness transitional care: a qualitative systematic review. *Clinical Nursing Research* 2022;31(2):163-173. DOI: 10.1177/10547738211056166. [Context Link 1, 2, 3, 4] View abstract...
15. Care coordination and care transitions to support adherence. In: Perez R, Rogers S, Prince M, Fraser K, editors. *Case Management Adherence Guidelines*. 2020 ed. Case Management Society of America; 2022:43-56. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10]
16. Monitoring and evaluation. In: Gillingham DC, editor. *Foundations of Case Management*. Blue Bayou Press; 2021:51-52. [Context Link 1, 2, 3, 4, 5, 6, 7, 8]
17. Bennett WL, et al. Strategies for Patient, Family, and Caregiver Engagement. Technical Brief Number 36. AHRQ Publication No. 20-EHC017 [Internet] Agency for Healthcare Research and Quality Effective Health Care Program. 2020 Aug Accessed at: <https://effectivehealthcare.ahrq.gov/>. [accessed 2023 Sep 13] DOI: 10.23970/AHRQEPCTB36. [Context Link 1]
18. Aboumatar H, et al. Patient engagement strategies for adults with chronic conditions: an evidence map. *Systematic Reviews* 2022;11(1):Online. DOI: 10.1186/s13643-021-01873-5. [Context Link 1] View abstract...
19. Hudon C, et al. Case management in primary care for frequent users of health care services: a realist synthesis. *Annals of Family Medicine* 2020;18(3):218-226. DOI: 10.1370/afm.2499. [Context Link 1] View abstract...
20. Psychiatric guidelines. In: Cash JC, editor. *Family Practice Guidelines*. 6th ed. Springer Publishing; 2023:888-923. [Context Link 1, 2, 3, 4]
21. Anger, hostility, and aggression. In: Videbeck SL, editor. *Psychiatric-Mental Health Nursing*. 9th ed. Wolters Kluwer; 2023:175-187. [Context Link 1]
22. Mood disorders and suicide. In: Videbeck SL, editor. *Psychiatric-Mental Health Nursing*. 9th ed. Wolters Kluwer; 2023:284-327. [Context Link 1, 2, 3, 4]
23. Physical Activity and Sedentary Behaviour. WHO Guideline [Internet] World Health Organization. 2020 Nov Accessed at: <https://www.who.int/publications/i/item/9789240015128>. [accessed 2023 Aug 14] [Context Link 1]
24. James AC, Reardon T, Soler A, James G, Creswell C. Cognitive behavioural therapy for anxiety disorders in children and adolescents. *Cochrane Database of Systematic Reviews* 2020, Issue 11. Art. No.: CD013162. DOI: 10.1002/14651858.CD013162.pub2. [Context Link 1, 2] View abstract...
25. Steele D. Working with an individual patient. In: Steele D, editor. *Keltner's Psychiatric Nursing*. 9th ed. Elsevier; 2023:83-97. [Context Link 1]
26. Psychopharmacology. In: Morgan KI, Townsend MC, editors. *Davis Advantage for Psychiatric Mental Health Nursing*. 10th ed. Philadelphia, PA: F.A. Davis; 2021:53-86. [Context Link 1, 2]
27. Anxiety, obsessive-compulsive, and related disorders. In: Morgan KI, Townsend MC, editors. *Davis Advantage for Psychiatric Mental Health Nursing*. 10th ed. Philadelphia, PA: F.A. Davis; 2021:547-577. [Context Link 1, 2, 3, 4, 5, 6]
28. Fosbre CD. Suicidal thoughts and behaviors. In: Fosbre CD, Varcarolis EM, Chiappetta L, editors. *Varcarolis' Essentials of Psychiatric-Mental Health Nursing*. 5th ed. Elsevier; 2023:377-387. [Context Link 1]
29. Neurologic care. In: Lynn P, editor. *Taylor's Clinical Nursing Skills*. 6th ed. Wolters Kluwer; 2023:1079-1113. [Context Link 1]
30. Depressive disorders. In: Morgan KI, Townsend MC, editors. *Davis Advantage for Psychiatric Mental Health Nursing*. 10th ed. Philadelphia, PA: F.A. Davis; 2021:479-518. [Context Link 1]
31. Smith CA. Renal and urologic problems. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:1195-1231. [Context Link 1]
32. Social Anxiety Disorder: Recognition, Assessment and Treatment. NICE Clinical Guidance CG159 [Internet] National Institute for Health and Care Excellence. 2013 May (NICE reviewed 2017) Accessed at: <https://www.nice.org.uk/guidance>. [accessed 2023 Aug 08] [Context Link 1, 2]
33. Cohen DJ, Davis MM, Hall JD, Gilchrist EC, Miller BF. A Guidebook of Professional Practices for Behavioral Health and Primary Care Integration. The Academy Integrating Behavioral Health and Primary Care AHRQ Publication No. 14-0070-1-EF [Internet] Agency for Healthcare Research and Quality. 2015 Mar Accessed at: <https://www.ahrq.gov/>. [accessed 2023 Aug 15] [Context Link 1]
34. Treatment settings and therapeutic programs. In: Videbeck SL, editor. *Psychiatric-Mental Health Nursing*. 9th ed. Wolters Kluwer; 2023:64-78. [Context Link 1]
35. The nursing process in drug therapy and patient safety. In: Tucker RG, editor. *Karch's Focus on Nursing Pharmacology*. 9th ed. Wolters Kluwer; 2023:47-57. [Context Link 1]
36. Antiepileptic agents. In: Tucker RG, editor. *Karch's Focus on Nursing Pharmacology*. 9th ed. Wolters Kluwer; 2023:396-417. [Context Link 1]
37. Anxiolytic and hypnotic agents. In: Tucker RG, editor. *Karch's Focus on Nursing Pharmacology*. 9th ed. Wolters Kluwer; 2023:342-357. [Context Link 1]
38. Drugs for bipolar disorder. In: Burchum JR, Rosenthal LD, editors. *Lehne's Pharmacology for Nursing Care*. 11th ed. Elsevier; 2022:372-379. [Context Link 1]
39. Psychotherapeutic agents. In: Tucker RG, editor. *Karch's Focus on Nursing Pharmacology*. 9th ed. Wolters Kluwer; 2023:376-395. [Context Link 1, 2, 3, 4]
40. Smith-East M, Keltner NL. Antipsychotic drugs. In: Steele D, editor. *Keltner's Psychiatric Nursing*. 9th ed. Elsevier; 2023:140-156. [Context Link 1]

41. Neurobiologic theories and psychopharmacology. In: Videbeck SL, editor. *Psychiatric-Mental Health Nursing*. 9th ed. Wolters Kluwer; 2023:15-40. [Context Link 1, 2]
42. Antipsychotic agents and their use in schizophrenia. In: Burchum JR, Rosenthal LD, editors. *Lehne's Pharmacology for Nursing Care*. 11th ed. Elsevier; 2022:334-350. [Context Link 1]
43. Reus VI. Psychiatric disorders. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. *Harrison's Principles of Internal Medicine*. 21st ed. McGraw Hill Education; 2022:3540-3556. [Context Link 1]
44. Management of anxiety disorders. In: Burchum JR, Rosenthal LD, editors. *Lehne's Pharmacology for Nursing Care*. 11th ed. Elsevier; 2022:395-401. [Context Link 1]
45. Alphabetical listing of drugs by generic name. In: Lippincott Williams & Wilkins, editor. *Nursing Drug Handbook*. 2024 ed. Wolters Kluwer Health; 2024:63-1581. [Context Link 1]
46. Keltner NL, Vossos H. Antidepressant drugs. In: Steele D, editor. *Keltner's Psychiatric Nursing*. 9th ed. Elsevier; 2023:157-176. [Context Link 1, 2]
47. Serotonin syndrome: what you should know to protect your patient. In: Lippincott Williams & Wilkins, editor. *Nursing Drug Handbook*. 2023 ed. Lippincott Williams & Wilkins; 2023:1604. [Context Link 1, 2]
48. Antidepressants. In: Burchum JR, Rosenthal LD, editors. *Lehne's Pharmacology for Nursing Care*. 11th ed. Elsevier; 2022:351-371. [Context Link 1, 2]
49. Selected therapeutic drug classifications. In: Lippincott Williams & Wilkins, editor. *Nursing Drug Handbook*. 2024 ed. Wolters Kluwer Health; 2024:23-62. [Context Link 1, 2]
50. Less commonly used drugs: indications and dosages. In: Lippincott Williams & Wilkins, editor. *Nursing Drug Handbook*. 2024 ed. Wolters Kluwer Health; 2024:1701-1790. [Context Link 1]
51. Antidepressant agents. In: Tucker RG, editor. *Karch's Focus on Nursing Pharmacology*. 9th ed. Wolters Kluwer; 2023:358-375. [Context Link 1]
52. Nicholson NR Jr. Social isolation. In: Larsen PD, editor. *Lubkin's Chronic Illness: Impact and Intervention*. 11th ed. Jones & Bartlett Learning; 2022:25-48. [Context Link 1]
53. Drake MA, Holston EC. Family as client. In: Rector C, Stanley MJ, editors. *Community and Public Health Nursing Promoting the Public's Health*. 10th ed. Wolters Kluwer; 2022:385-415. [Context Link 1, 2]
54. Scanlon A, Reinisch C. Social determinants of health. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:18-20. [Context Link 1, 2]
55. Todd D. Health promotion through education. In: Rector C, Stanley MJ, editors. *Community and Public Health Nursing Promoting the Public's Health*. 10th ed. Wolters Kluwer; 2022:305-331. [Context Link 1]
56. Mulligan K, Bhatti S, Rayner J, Hsiung S. Social prescribing: creating pathways towards better health and wellness. *Journal of the American Geriatrics Society* 2020;68(2):426-428. DOI: 10.1111/jgs.16249. [Context Link 1] View abstract...
57. Greenberg SA. Chronic illness and older adults. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:63-80. [Context Link 1]
58. Transition Between Inpatient Hospital Settings and Community or Care Home Settings for Adults With Social Care Needs. NICE Quality Standard QS136 [Internet] National Institute for Health and Care Excellence. 2016 Dec Accessed at: <https://www.nice.org.uk/guidance/>. [accessed 2023 Aug 08] [Context Link 1]
59. A - W. In: Phelps LL, editor. *Nursing Diagnosis Reference Manual*. 12th ed. Wolters Kluwer; 2023:2-750. [Context Link 1, 2]
60. Greenough GP, Judd BG. Sleep disorders. In: Buttaro TM, Polgar-Bailey P, Sandberg-Cook J, Trybulski JA, editors. *Primary Care: A Collaborative Practice*. 6th ed. St. Louis, MO: Elsevier; 2021:1252-1262. [Context Link 1, 2]
61. Gold R. Insomnia. In: Ferri FF, editor. *Ferri's Clinical Advisor*. 2024 ed. Elsevier; 2024:804.e13-804.e22. [Context Link 1, 2]
62. Sateia MJ, Buysse DJ, Krystal AD, Neubauer DN, Heald JL. Clinical practice guideline for the pharmacologic treatment of chronic insomnia in adults: an American Academy of Sleep Medicine clinical practice guideline. *Journal of Clinical Sleep Medicine* 2017;13(2):307-349. DOI: 10.5664/jcsm.6470. (Reaffirmed 2023 Jun) [Context Link 1, 2] View abstract...
63. Weilburg JB, Winkelman J. Approach to the patient with insomnia. In: Goroll AH, Mulley AG Jr., editors. *Primary Care Medicine: Office Evaluation and Management of the Adult Patient*. 8th ed. Philadelphia, PA: Wolters Kluwer; 2021:1760-1771. [Context Link 1, 2, 3]
64. Appendix A: Disorders of sleep and wakefulness. In: Videbeck SL, editor. *Psychiatric-Mental Health Nursing*. 9th ed. Wolters Kluwer; 2023:483-485. [Context Link 1]
65. Sejbuk M, Mironczuk-Chodakowska I, Witkowska AM. Sleep quality: a narrative review on nutrition, stimulants, and physical activity as important factors. *Nutrients* 2022;14(9):Online. DOI: 10.3390/nu14091912. [Context Link 1] View abstract...
66. Mansukhani MP, Kolla BP, St. Louis EK. Sleep disorders. In: Kellerman RD, Rakel DP, Heidelbaugh JJ, Lee EM, editors. *Conn's Current Therapy*. 2023 ed. Philadelphia, PA: Elsevier; 2023:794-809. [Context Link 1, 2]
67. Edinger JD, et al. Behavioral and psychological treatments for chronic insomnia disorder in adults: an American Academy of Sleep Medicine clinical practice guideline. *Journal of Clinical Sleep Medicine* 2021;17(2):255-262. DOI: 10.5664/jcsm.8986. [Context Link 1] View abstract...
68. Cadena-Malek Y. Communicable disease. In: Rector C, Stanley MJ, editors. *Community and Public Health Nursing Promoting the Public's Health*. 10th ed. Wolters Kluwer; 2022:204-241. [Context Link 1]
69. Murthy N, Wodi AP, McNally V, Cineas S, Ault K. Advisory committee on immunization practices recommended immunization schedule for adults aged 19 years or older - United States, 2023. *MMWR--Morbidity and Mortality Weekly Report* 2023;72(6):141-144. DOI: 10.15585/mmwr.mm7206a2. [Context Link 1] View abstract...

70. Recommended Adult Immunization Schedule for Ages 19 Years or Older, United States. Immunization Recommendations, United States, 2023 [Internet] Centers for Disease Control and Prevention. Accessed at: <https://www.cdc.gov/vaccines/schedules/hcp/imz/adult.html>. Updated 2023 Feb [accessed 2023 Oct 11] [Context Link 1, 2]
71. Bass AR, et al. 2022 American College of Rheumatology guideline for vaccinations in patients with rheumatic and musculoskeletal diseases. *Arthritis Care & Research* 2023;75(3):449-464. DOI: 10.1002/acr.25045. (Reaffirmed 2023 Mar) [Context Link 1] View abstract...
72. Sarasnick JA. Upper respiratory problems. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:577-595. [Context Link 1]
73. Standards of Practice for Case Management. [Internet] Case Management Society of America. 2022 Accessed at: <https://www.cmsa.org/>. [accessed 2023 Sep 21] [Context Link 1, 2, 3, 4, 5]
74. Strawn JR, Peris TS, Walkup JT. Anxiety disorders. In: Dulcan MK, editor. *Dulcan's Textbook of Child and Adolescent Psychiatry*. 3rd ed. American Psychiatric Association Publishing; 2022:309-344. [Context Link 1]
75. Accreditation bodies, their standards, and requirements. In: Gillingham DC, editor. *Foundations of Case Management*. Blue Bayou Press; 2021:241-247. [Context Link 1, 2]
76. Standards and Guidelines for the Accreditation and Certification for Case Management Accreditation. Effective for Surveys Beginning on or After July 1, 2020 [Internet] National Committee for Quality Assurance (NCQA). 2020 Accessed at: To purchase copies of this publication, visit <https://ncqa.org>. [accessed 2023 Oct 24] [Context Link 1, 2]
77. Standards and Guidelines for the Accreditation of Case Management for Long-Term Services and Supports Program. Effective for Surveys Beginning on or After July 1, 2024 [Internet] National Committee for Quality Assurance (NCQA). 2023 Accessed at: To purchase copies of this publication, visit <https://ncqa.org>. [accessed 2023 Sep 05] [Context Link 1, 2]
78. Physical Activity Guidelines for Americans. [Internet] U.S. Department of Health and Human Services. 2nd ed; 2018 Accessed at: <https://health.gov/our-work>. [accessed 2023 Sep 18] [Context Link 1]
79. Nall RW, Herndon B, Mramba LK, Vogel-Anderson K, Hagen MG. An interprofessional primary care based transition of care clinic to reduce hospital readmission. *American Journal of Medicine* 2020;133(6):e260-e268. DOI: 10.1016/j.amjmed.2019.10.040. [Context Link 1] View abstract...
80. Nguyen HQ, et al. Association between post-hospital clinic and telephone follow-up provider visits with 30-day readmission risk in an integrated health system. *BMC Health Services Research* 2021;21(1):826. DOI: 10.1186/s12913-021-06848-9. [Context Link 1] View abstract...
81. Anderson A, et al. Follow-up post-discharge and readmission disparities among Medicare fee-for-service beneficiaries, 2018. *Journal of General Internal Medicine* 2022;37(12):3020-3028. DOI: 10.1007/s11606-022-07488-3. [Context Link 1] View abstract...
82. Marcondes FO, et al. Does scheduling a postdischarge visit with a primary care physician increase rates of follow-up and decrease readmissions? *Journal of Hospital Medicine* 2019;14:E37-E42. DOI: 10.12788/jhm.3309. [Context Link 1] View abstract...
83. Coppa K, Kim EJ, Oppenheim MI, Bock KR, Conigliaro J, Hirsch JS. Examination of post-discharge follow-up appointment status and 30-day readmission. *Journal of General Internal Medicine* 2021;36(5):1214-1221. DOI: 10.1007/s11606-020-06569-5. [Context Link 1] View abstract...
84. Bethishou L, Herzik K, Fang N, Abdo C, Tomaszewski DM. The impact of the pharmacist on continuity of care during transitions of care: A systematic review. *Journal of the American Pharmacists Association* 2020;60(1):163-177.e2. DOI: 10.1016/j.japh.2019.06.020. [Context Link 1] View abstract...
85. Medalie L, Cifu AS. Management of chronic insomnia disorder in adults. *Journal of the American Medical Association* 2017;317(7):762-763. DOI: 10.1001/jama.2016.19004. [Context Link 1] View abstract...
86. Milieu therapy - the therapeutic community. In: Morgan KI, Townsend MC, editors. *Davis Advantage for Psychiatric Mental Health Nursing*. 10th ed. Philadelphia, PA: F.A. Davis; 2021:196-206. [Context Link 1]
87. Gentry MT, Lapid MI, Rummans TA. Geriatric telepsychiatry: systematic review and policy considerations. *American Journal of Geriatric Psychiatry* 2019;27(2):109-127. DOI: 10.1016/j.jagp.2018.10.009. [Context Link 1] View abstract...
88. The case management process. In: Powell SK, Tahan H, editors. *Case Management a Practical Guide for Education and Practice*. 4th ed. Philadelphia, PA: Wolters Kluwer, Lippincott, Williams & Wilkins; 2019:92-118. [Context Link 1]
89. Szuhany KL, Simon NM. Anxiety disorders: a review. *Journal of the American Medical Association* 2022;328(24):2431-2445. DOI: 10.1001/jama.2022.22744. [Context Link 1] View abstract...

Footnotes

[A] Assess and address the emergency or urgent problems before continuing the engagement. [A in Context Link 1]

[B] A self-care plan tells you how to take care of your condition and when and where to get help for problems. [B in Context Link 1]

[C] Psychosocial interventions should be individualized and involve family and other supports as necessary.(8) [C in Context Link 1]

[D] Referral to a behavioral health specialist is based on an established treatment plan and needs at discharge from inpatient care. A psychiatrist serves as the leader of the inpatient team and can perform psychotherapy, medication prescription, and other somatic therapies. A clinical psychologist conducts individual, group, and family therapy, and administers and interprets psychological testing. A psychiatric social worker conducts individual, group, and family therapy addressing a patient's social needs to include placement, financial support, and community requirements.(86) [D in Context Link 1]

- [E] Telehealth may improve access to and coordination of mental health care. Patients with behavioral health diagnoses may use telehealth or telepsychiatry to access mental health support or care (eg, to get assistance with problem-solving techniques or ask about presenting symptoms or medication use).(87) [E in Context Link 1]
- [F] The timing of provider follow-up depends on the patient's disease severity, self-care complexity, patient/caregiver willingness and ability to provide patient care, and available social supports(88) [F in Context Link 1]
- [G] Follow-up is recommended every 2 to 4 weeks for the first 3 months, then every 3 months to assess the patient's status.(20) [G in Context Link 1]
- [H] The prevalence of either alcohol use or substance use disorder is approximately 16.5% over a 12-month period in patients with at least one anxiety disorder.(89) [H in Context Link 1]
- [I] Neuroleptic malignant syndrome is a rare, rapidly progressive, potentially fatal complication more common with typical antipsychotics. Onset can occur hours or years after initiation of drug therapy.(41) [I in Context Link 1]

Codes

ICD-10 Diagnosis: F06.4, F10.180, F10.280, F10.980, F12.180, F12.280, F12.980, F13.180, F13.280, F13.980, F14.180, F14.280, F14.980, F15.180, F15.280, F15.980, F16.180, F16.280, F16.980, F18.180, F18.280, F18.980, F19.180, F19.280, F19.980, F34.1, F40.10, F40.11, F40.8, F40.9, F41.0, F41.1, F41.3, F41.8, F41.9, F43.22, F43.23, F93.0, R45.88, Z91.51, Z91.52 [Hide]

DSM-5: F06.4, F10.180, F10.280, F10.980, F12.180, F12.280, F12.980, F13.180, F13.280, F13.980, F14.180, F14.280, F14.980, F15.180, F15.280, F15.980, F16.180, F16.280, F16.980, F18.180, F18.280, F18.980, F19.180, F19.280, F19.980, F34.1, F40.10, F41.0, F41.1, F41.8, F41.9, F43.22, F43.23, F93.0, R45.88 [Hide]

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